



County of Santa Clara
Emergency Medical Services
Policies and Procedures

Policy Number:	410
Effective Date:	1/1/2026
Scheduled Review:	12/1/2029

PEDIATRIC RECEIVING CENTER STANDARDS

- I. **Purpose:** Provide the procedures and standards for the Santa Clara County EMS for Children (EMSC) Care System. This system links pre-hospital and hospital care to ensure pediatric patients are triaged and prioritized to the most appropriate pediatric receiving center and receive optimal care based on these standards.
- II. **Authority:** Health and Safety Code, Division 2.5, Sections [1798.150](#), [1799.202-204](#), [1797.220](#); California Code of Regulations, Title 22, Division 9, [Chapter 6.4](#)
- III. **Policy:**
- A. **The Santa Clara County EMS Agency (EMS Agency)**
- The EMS Agency and EMS Medical Director are responsible for the oversight and management of the EMS for Children System including but not limited to planning, implementation and evaluation. Areas of responsibility include:
 - Designation and monitoring for compliance with standards.
 - Medical control, clinical operations, training and management of the EMS for Children system in the pre-hospital care setting.
 - Developing pre-hospital screening, triage and destination policies.
 - Coordinating EMS for Children system quality improvement.
- B. **Pediatric Receiving Center Designation Levels:**
- A **General Pediatric Receiving Center (General PedRC)** is an EMS-designated facility equipped to provide basic emergency care for pediatric patients. These centers are staffed with providers trained in pediatric assessment and follow established protocols for rapid assessment, stabilization, and transfer when higher levels of pediatric care are required. General PedRCs serve as entry points for pediatric emergencies within the EMS system.
 - Advanced Pediatric Receiving Centers**
 - An **Advanced Pediatric Receiving Center (Advanced PedRC)** is an EMS-designated facility capable of providing emergency stabilization and initial management for critically ill or injured pediatric patients. It maintains pediatric-trained staff and resources to deliver timely, age-appropriate emergency and definitive care, with established protocols but may need to transfer specialty cases to higher-level centers as needed.
 - A **Comprehensive Pediatric Receiving Center (Comprehensive PedRC)** is a higher-level, EMS-designated pediatric facility equipped to provide definitive care for all critically ill or injured children, including those requiring intensive care or specialty surgical services. Unlike an Advanced Pediatric Receiving Center, which focuses on emergency stabilization and initial management, a Comprehensive PedRC offers 24/7 in-house pediatric subspecialists, and advanced diagnostic and therapeutic capabilities, ensuring full-spectrum pediatric care. Comprehensive PedRCs serve as a regional referral center for specialized care within the EMS System.
- IV. **Procedure**
- A. **Designation**
- The process for initial designation as a Pediatric Receiving Center by Santa Clara County Emergency Medical Services is detailed in [Policy 400](#).
 - Continued designation as a Pediatric Receiving Center (PedRC) shall depend on:
 - Current license as a general acute care hospital with a basic or comprehensive emergency department permit.

APPROVAL:

Signature on File

Nick Clay, EMS Director/Chief

Signature on File

Nicole D'Arcy MD, EMS Medical Director

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- b. Current written agreement with the EMS Agency for designation as a Pediatric Receiving Center.
- c. Currently serving in the EMS System as a Pre-hospital Receiving Center (PRC).
- d. Compliance with all State regulations and Santa Clara County EMS Agency policies and procedures including those detailed herein.
- e. Advanced/Comprehensive PedRCs shall meet California requirements for neonatal intensive care unit (NICU) and pediatric intensive care unit (PICU) licensing and certification.

B. Pediatric Receiving Center Standards

1. A designated PedRC shall meet all the following standards:
 - a. Hospital Facility and Services
 - i. The ED shall be able to stabilize critically ill or injured infants, children and adolescents prior to transfer to an Advanced PedRC or admission to NICU or PICU.
 - ii. At a minimum, one licensed registered nurse or advanced care practitioner per shift in the emergency department with a current nationally recognized Advanced Pediatric Life Support course such as PALS/APLS/ Emergency Nursing Pediatric Course.
 - iii. Maintain the minimum equipment listed in Attachment A of this policy.
 - iv. A process for obtaining and providing consultation via phone, telehealth, or onsite for emergency care, stabilization, transfer and transport.
 1. Consult personnel must include a qualified pediatric specialist and a pediatric intensivist.
 - v. Have policies and procedures for interfacility transfers of pediatric patients that include the requirements listed in State regulations and align with current EMS Agency transfer policies.
 1. General PedRCs shall have established agreements with an Advanced and/or Comprehensive PedRC for consultation and transfer.
 2. An Advanced PedRC shall have established an agreement with at least one Comprehensive PedRC for consultation and transfer of pediatric patients needing specialized care such as trauma, burn, spinal cord injury, rehabilitation, and behavioral health.
 - vi. Advanced/Comprehensive PedRCs shall have both a PICU and NICU.
 - vii. Advanced/Comprehensive PedRCs shall have qualified specialists available as detailed in Attachment B of this Policy.
 - viii. Comprehensive PedRC shall have ED services that include a separate pediatric ED or a designated area for emergency care of pediatric patients within the department and physician staff who are qualified specialists in emergency medicine or pediatric emergency medicine.
 - b. Support Services
 - i. All support services listed below must have equipment and staff trained for the care of pediatric patients.
 1. Respiratory Care Specialists who respond to the emergency department.
 - a. Must have current nationally recognized Advanced Pediatric Life Support course such as PALS/APLS.
 - b. Must have annual continuing education and competencies specific to the resuscitation of pediatric patients.
 2. Laboratory Services
 3. Radiology
 4. Pharmacy
 - ii. Additional support personnel requirements for Advanced/Comprehensive PedRC include:
 1. Social work services provided by a medical social worker (MSW) holding a master's degree in social work and has expertise in issues affecting families of seriously ill pediatric patients.
 2. Behavioral health specialists with pediatric experience including but not limited to psychiatrists, psychologists, and nurses.
 - c. Program Personnel

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- i. Have two designated Pediatric Emergency Care Coordinators (PECC), one shall be a physician and the other may be a nurse, a nurse practitioner, or Physician Assistant who is assigned to the ED or has emergency care responsibility and meets the following requirements:
 1. Physician PECC
 - a. Licensed in California, be a board-certified emergency medicine physician, or qualified, specialist in Pediatrics or Family Medicine, and
 - b. Maintain competency in resuscitation of neonatal and pediatric patients through a nationally recognized Advanced Pediatric Life Support course such as PALS/APLS.
 2. Nurse, nurse practitioner, and physician assistant PECC
 - a. Licensed in California, have at least two (2) years of experience in pediatric or emergency nursing within the previous five (5) years and maintain
 - b. Maintain competency in resuscitation of neonatal and pediatric patients through a nationally recognized Advanced Pediatric Life Support course such as PALS/APLS.
 - c. Complete a minimum of ten (10) continuing education contact hours of pediatric emergency care every two years.
- ii. The designated PECC is responsible for providing oversight of pediatric quality improvement initiatives within the emergency department, serving as a liaison with internal and external pediatric care stakeholders, facilitating continuing education and competencies in pediatric emergency care for ED staff, coordinating pediatric disaster preparedness, and promoting family-centered care practices.
- iii. General PedRCs may share the Physician PECC with other PedRCs.
- d. EMSC System Participation
 - i. Participate in the pre-hospital performance improvement process and offer expertise to the EMS Medical Director in the development of pre-hospital care protocols for pediatric patients.
 - ii. Participate in the EMS for Children Quality Improvement Committee (EMSCQIC) as per [Policy 411](#) and other related committees when requested.
 - iii. Participate in the pre-hospital quality improvement process through selected review of pediatric related, deaths, major complications and interfacility transfers when requested by the EMS Agency.
 1. The designated PECC will work with the EMS Agency Specialty Programs Nurse Coordinator to obtain the necessary chart materials for the medical review process and committee meetings.
- e. Data Collection and Submission
 - i. The EMS Agency will be responsible for creating a Santa Clara County EMSC Data Dictionary to include required local and state data elements and will be updated annually.
 - ii. PedRCs shall submit validated, error-free data in an agreed-upon format to the EMS Agency on regularly assigned dates.
 - iii. The EMS Agency will submit pediatric data to the California EMS Information System (CEMSIS) as required by State regulations.
- f. Performance Standards
 - i. All PedRCs shall meet the following performance standards:
 1. Complete an online assessment of the National Pediatric Readiness Project self-assessment and share the results with the EMS Agency annually.
 2. Have a process to review and revise facility pediatric policies and protocols at least every three (3) years.
 3. Have a data-driven quality improvement program that monitors standardized clinical performance measures which align with state and national standards.
 4. Additional performance measures may be determined by the EMS Agency and/or the EMSCQIC.
 - ii. Have a multidisciplinary QI committee to review pediatric cases that include:

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1. Cardiopulmonary or respiratory arrests
 2. Child maltreatment cases
 3. Deaths
 4. Intensive care unit admissions in regard to length of stay
 5. Operating room admissions and type of surgery
 6. Transfers and ED boarding
- g. Education
- i. All PedRCs shall develop continuing education, competencies, and drills/mock codes, and disaster exercises to ensure staff physicians, nurses, and support services staff receive annual pediatric specific education. The training plan shall be submitted to the EMS Agency annually.
 - ii. Advanced/Comprehensive PedRCs must meet the following additional education requirements:
 1. Provide pre-hospital pediatric emergency care continuing education opportunities at least once per year for EMS personnel and report these activities to the EMS Agency by the end of each calendar year.
 2. Provide pediatric emergency care education and guidance to General PedRCs in which they maintain transfer agreements with.
 3. Have a community education and outreach campaign in collaboration with the EMS Agency and General PedRCs. Report activities to the EMS Agency by the end of each calendar year.

V. Special Considerations

- A. Reporting Requirements
1. Shall notify the EMS Agency in writing of any failure to meet these Pediatric Receiving Center Standards within 48 hours.
 2. Changes to program personnel shall be reported to the EMS Agency within ten (10) business days:
 - a. Designated Physician PECC
 - b. Designated Nurse PECC
- B. Pediatric Receiving Center Equipment Requirements

Pediatric Receiving Center Equipment Requirements
System/process to assure proper sizing of resuscitation equipment and proper dosing of medication (length-based tape, medical software, etc.)
Size appropriate monitoring equipment: BP cuffs, pulse oximeter, end tidal CO2, doppler, thermometer, ECG monitor, defibrillator
Portable resuscitation supplies & method of verification of contents on a regular basis. (crash cart, bag, box etc.). Must be available in ED, radiology suite and all inpatient pediatric units
Resuscitation medications
Equipment for patient and fluid warming
Weight scale (in kilograms)
Age-appropriate pain scales
Patient restraints & policy for use
Size appropriate respiratory equipment: clear O2 masks, bag-mask devices, intubation equipment, supraglottic airways, oral & nasal airways, nasogastric tubes, suction equipment
Size appropriate vascular access supplies (IV, IO, infusion devices, IV solutions)
Fracture management devices (extremity splints, spinal motion restriction, traction)
Size appropriate lumbar puncture trays
Size appropriate difficult airway kits
Tube thoracostomy tray including size appropriate chest tubes
Urinary catheter tray including size appropriate urinary catheters

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Newborn delivery kit including a towel, clamps and scissors for cutting the umbilical cord, bulb suction, warming pad, neonatal bag-mask ventilation device with appropriately sized masks.

Neonatal resuscitation equipment including:
 pediatric laryngoscope with Miller
 0 and 00 blades, size 2.5 and 3.0 endotracheal tubes, and umbilical vein catheters

C. Advanced Pediatric Receiving Center Personnel Coverage Requirements

Advanced Pediatric Receiving Center Personnel Coverage Requirements	
The following qualified specialist will be available twenty-four (24) hours a day, seven (7) days a week, for consultation by phone, telehealth, or transfer agreement: Pediatric Gastroenterologist Pediatric Hematologist/Oncologist Pediatric Infectious Disease Pediatric Nephrologist Pediatric Neurologist Pediatric Surgeon Cardiac Surgeon with pediatric experience Neurosurgeon with pediatric experience Obstetric/Gynecologist with pediatric experience Pulmonologist with pediatric experience Pediatric Endocrinologist	The following specialist will be on call, and available to the ED/PICU/NICU either in-person, by phone, or telehealth, within thirty (30) minutes: Radiologist with pediatric experience Otolaryngologist with pediatric experience Orthopedist with pediatric experience Mental health professional with pediatric experience
	The following specialties shall be on-call and available for consultation to the ED/PICU/NICU within 30 minutes by phone and in-person within one (1) hour: General Surgeon with pediatric experience Anesthesiologist with pediatric experience Pediatric Cardiologist Neonatologist

I. References:

- A. National Pediatric Readiness Project. Pediatric Readiness Assessment. Retrieved June 27, 2025, from <https://www.pedsready.org/>
- B. EMS Policy 101-Definitions

II. Attachments: None